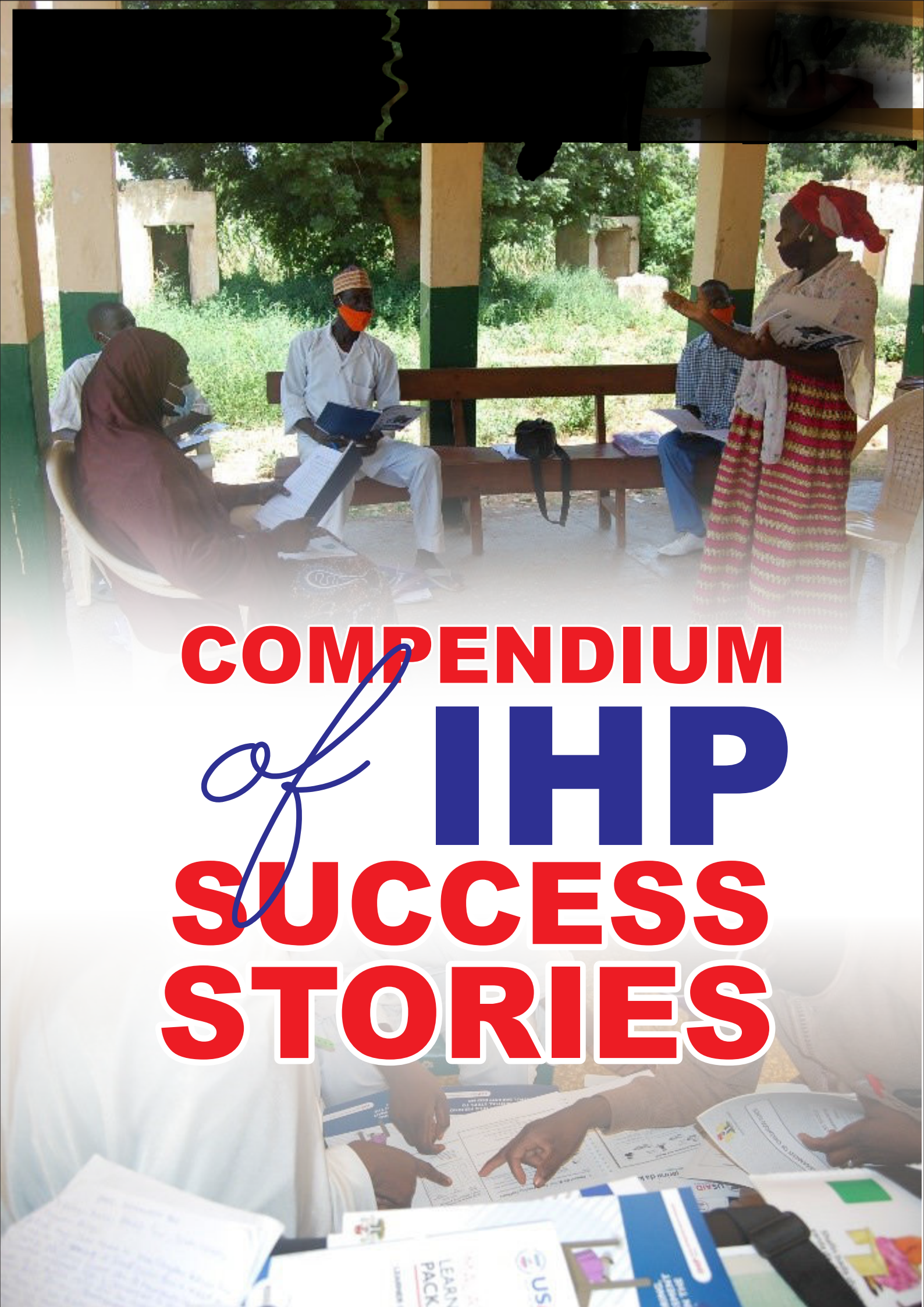




COMPENDIUM *of* **IHP** **SUCCESS** **STORIES**

INTRODUCTION

Globally, infectious diseases, including pneumonia, diarrhea and malaria, along with pre-term birth, birth asphyxia and trauma, and congenital anomalies remain the leading causes of death for children under five. Access to basic lifesaving interventions such as skilled delivery at birth, postnatal care, breastfeeding and adequate nutrition, vaccinations, and treatment for common childhood diseases can save many young lives. Malnourished children, particularly those with severe acute malnutrition, have a higher risk of death from common childhood illness such as diarrhea, pneumonia, and malaria. Nutrition-related factors contribute to about 45% of deaths in children under-5 years of age. USAID through the Integrated Health Program understands this and have trained capable hands to facilitate, a Facility based clinical skills training for healthcare providers for the delivery of integrated child health, nutrition, malaria, and routine immunization modular training packages using the LDHF approach in Sokoto State, Kebbi & Bauchi State.

Below is a compilation of interaction with trainees and trainers in Sokoto and Kebbi States;

SOKOTO STATE

CHIMOLA HEALTH POST

Change they say is the only constant thing in life, the perception of the Health Workers in Chimola HP has taken a positive turn; Aishatu Ahmed, Zayyanu Umar S. Gobir, Murtala Jelani, Bello Aliyu and Ahmed Musa in the time past do treat children who they confirm has Malaria with Chloroquine. However, the training has made them understand that ACT is the right drug to treat Malaria, they also didn't know how to counsel patients properly but now they have become





professional counsellors as they take time to explain to Caregivers what they need to know and they can overcome sicknesses by practicing personal and environmental hygiene. They mentioned that the Chart Booklet has also been helpful in treating young children as they use the booklet as a guide and follow all the right steps. Aishatu spoke on how she can now easily identify danger signs when she sees one, she mentioned that it was initially difficult to differentiate between Cough, Cold & Pneumonia.

Thankfully, she can now differentiate between the 3 sicknesses and give the right treatment, she mentioned that she now knows there are cases they can treat and the ones they can't treat in their Centre. She said the training has shaped them mentally and they have seen changes in how they carry out their duties professionally.

SALAME PHC GWADABAWA

On interacting with trainees from this facility, they reported that they do not

know how to do proper counselling but now they know how to go about it and that has improved the turn-out of patients and caregivers as they now themselves seek medical advice on matters that confuse them. They also mentioned that they have added knowledge on Illnesses that affect children generally.



After the training they now know how to assess, classify and treat children using the Chart Booklet as a guide to treat and the Sick Child Recording Form to fill the necessary details and ask the right questions that will make treatment easier and faster. They spoke on added knowledge on the prevention of Malaria; this include sleeping in LLIN, spraying the



premises with insecticides and mosquito repellants, wearing long garments to sleep and by keeping the surrounding clean.

TAMBAGARKA PHC



This training was indeed an eye opener to Bello Shehu, Tamiu Haliu, Suraju Shehu, Ismail Abubakar K. and Hanatu Yakubu. They explained that they do give polio twice but they now do so three times during immunization session. They also said they treated Malaria blindly before

now without carrying out any test, thanks to the LDHF training they now confirm the status of the child using RDT before carrying out any form of treatment. On Nutrition, they explain to lactating and expecting mothers the importance of Exclusive Breast feeding and why the first Brest Milk is important as it contains Colostrum that helps the baby grow and enhances the baby's knowledge. They also said they now make use of their weighing scale and height ruler which was usually abandoned without use, they spoke well of the training and expressed their joy as their knowledge has been duly enhanced in key areas of child health.

The trainer added that she is happy that the trainees have been putting to use what they have learnt from the training as she has seen great improvement in the way they



handle cases, how they counsel Caregivers and treat children. Tamiu mentioned that he didn't know Diarrhea is in classes and that not all kinds can be treated at their level.

MAMMADE HEALTH POST

During my visit to the health post, Rukaya Shehu, Kasim Ahmed, Gazali Saadu and Aliyu Abdullahi were available.

They said initially they were not taking the weight of a baby to ascertain the child's nutritional status, and they do not make use of the thermometer and that the RDT is not done properly.

After the training they were able to know weight helps in the kind of drugs and dosage to administer, they talked on the importance of using MUAC Tape so as to give the right nutritional advices to Caregivers. Now, they give priority to RDT before prescribing drugs to clients so as to know the type of fever. They now know the cases to manage and those to refer, they give pre-referral treatment to cases beyond them before referring. Rukaya mentioned that they now do proper documentation and their overall



Health Management System has been improved as they now treat Diarrhea patients with ORS & Zinc.



The trainer spoke highly of how the training has changed the life of her trainees as in previous times they treat children with Chloroquine injection but through what was taught she has succeeded in convincing them to use the right drugs such as; ACT.

GIDAN DIYA PHC



In Bashar's words "before now we do treat patients diagnosed of Diarrhea with Antibiotics instead of Zinc, we were ignorant of this but the training has enhanced our knowledge. Now we know Diarrhea has 3 plans". In the aspect of Nutrition our knowledge has been enhanced on; Exclusive Breastfeeding for 6 months, types of Complementary feeding, healthy foods for lactating mothers, the proper use of the MUAC Tape, weighing scale and Height ruler to ascertain the nutritional status of the child so the right advise can be given to the caregiver.

In treating malaria, we ask questions first, check the temperature with a thermometer. On visiting this facility Hindatu A. Faruk, Jummai Adamu, Jamila Abdul Wahab and Rabi Ibrahim were available, during the

and use the MRDT to conduct test and if positive treat with ACT. The weight of the child also helps, if the result of the MRDT came out negative we check out for signs of infection or fever and give the right treatment. Lastly, we now make good use of the Chart Booklet and that helps us in treating right and saves time unlike before when we just treat randomly.

GUMBI HEALTH CENTRE



interaction they mentioned that before now when a patient is brought, they just ask what's wrong and prescribe drugs.



However, the training has shaped their mentality as they now listen, feel, assess and then classify the kind of illness before treating the patient. They also said they do not know about the danger signs and found it difficult to classify childhood illnesses. Thanks to the training, we can now identify a convulsing child, one with Diarrhea and they can now identify the severity of the illness, this made them know when to refer and when not to refer unlike in previous times where they try to treat every case.

Jummai mentioned that when a child is diagnosed of Malaria, they just prescribe Quinine for that child but now they know ACT, Salbutamol are the right drugs to issue. The trainees further said they do not use any document as a guide but now the Chart Booklet guides their every move as they know when a case is in the Green

section they are to advise the Caregiver, when it is in the Yellow section it is mild and they can treat at their facility; while when a case is in the Pink line it is severe and they are to give prereferral treatment before referring to a secondary Health Care center.

WAMMAKKO PHC



Indeed, knowledge is a key tool to development, before now we just observe the signs and symptoms and prescribe drugs. We do not carry out proper testing, the Chart Booklet has helped us a lot and has made our work

easier. We now fill the Sick Child Recording Form and follow the due process, the RDT kit is used to know if a child test positive or negative to malaria before knowing the next line of action. For a child with Diarrhea we now do intense



questioning, we ask how many times he stools, the nature of the stool before prescribing the right drugs. During counselling we tell Caregivers that Malaria can be prevented if only they sleep in LLIN, wear long garments, do proper environmental sanitation and disposing water properly.

Undeniably, we have seen major changes even in our people, most women believe the first breast milk is dangerous to the health of the baby, but the training has opened our eyes to the importance of the first breast milk as it contains colostrum and that is highly nutritious for the baby.

The trainer also mentioned that the training has helped the trainees a lot as they are more educated on Child Management, the right dosage of antimalaria as they just estimate in the past.

BINJI HEALTH POST



We didn't know not all illnesses should be treated here, now we know there are cases that can be managed at home, some at the HP here and others at a higher level. In the past whenever a child has cough, we just prescribe

a cough syrup, this training has made us understand that palm oil & sugar can be used to cure cough at home. On Malaria, we just issue chloroquine but now we prescribe ACT and we know the right dose to give, the RDT kit has also tremendously helped us in treating Malaria without much stress. We can now classify complicated and uncomplicated malaria. During Antenatal session we enlighten expecting mothers on Exclusive B/feeding and the right kind of foods to consume so as to be healthy and have enough breastmilk, those foods include; fruits, Milk, eggs, rice,



grains and yam. During Immunization sessions, we weigh the baby so as to check for changes and give the right counsel, when a child is malnourished, we make use of the MUAC tape. This training has changed our professional & even our personal lives.

The trainer added that she has noticed the trainees can now use the Chart Booklet appropriately and their counselling skills has leveled up, she mentioned that they make use of the ANC sessions to counsel pregnant mothers on Malaria and its dangers.

JAMALI PHC

At this facility Bello Danhadi, Isiaka Muhd and Doris Okereke were met and they spoke extensively on Exclusive Breast feeding and complementary feeding for children. They said their usage of the MUAC Tape wasn't so apt as they

didn't understand how to use it, thanks to the training they now know how to make use of the tape to know the level of the



child's nutritional status. They also said they can now treat Malaria as not all fever is malaria fever, they didn't know the various classifications of Malaria and just treated all the same way, after carrying out the Rapid Diagnostic Test using the available Kit, they wait for the result to be sure it's malaria and they prescribe Artesunate. They mentioned that the chart booklet has helped them do the proper treatment as they follow it appropriately and do no longer treat from their experience or by just seeing symptoms.

The trainer added that her trainees have improved knowledge in the area of Child Health Care and proper management as they have improved greatly compared to their first encounter.

KEBBI STATE

PHC DAKIN/GARI

At this facility Bashar Abubakar, Zainab Muhammad and Abubakar Usman were met, they spoke in details and said

before now they treat children based on experience but now, they have a guide that instructs and they follow duly, they mentioned that whatever is beyond the Chart Booklet they refer to the nearest institution. When we have cases of Malaria in the past, we always try to treat here at our level no matter how severe the case is, this training has been very helpful to us cause now once we carry out test using the RDT and we discover the Malaria Fever is severe we give pre-referral treatment and refer immediately. Secondly, the training has helped us manage cases of children less than 5 years of age, cases of under 5 has always been complicated, we have now learnt home remedies that work easier for the children and we now treat them more tenderly and this has saved us time



and made our work easier. We use the Child Growth Monitoring to monitor the child's growth even during Routine

Immunization, we now have added knowledge on the right dosage of drugs such unlike in the past when we give dosages according to our experience and the symptoms.

Another good thing is the Digital Weighing scale given to use, we had the manual and it was quite tasking to use and this made us a little lax when it's time to make use of the scale, the digital is faster, more accurate and saves time.



Lastly, Bashar said, when using the RDT they use 2-3 drops not minding the leaflet there in, the training has opened their eyes to always read the leaflet and follow whatever it says as every manufacturer has his own rules. He also mentioned that ever since they have been doing this the MRDT result has been accurate.

PHC BARBAREJO



At Barbarejo, the trainees spoke on how they didn't care who brings a child for immunization as long as the child is around. Now they have learnt the mother of child they should mainly bring the child for immunization

so, they can be exposed to the dangers of not completing the routine immunization and the advantages. They also, spoke on properly using and managing the weighing scale and MUAC Tape (what the colors on



it indicate, how it should be placed and when to use). They mentioned that their work ethics has improved greatly and they now make good use of all the equipment available for efficient results.

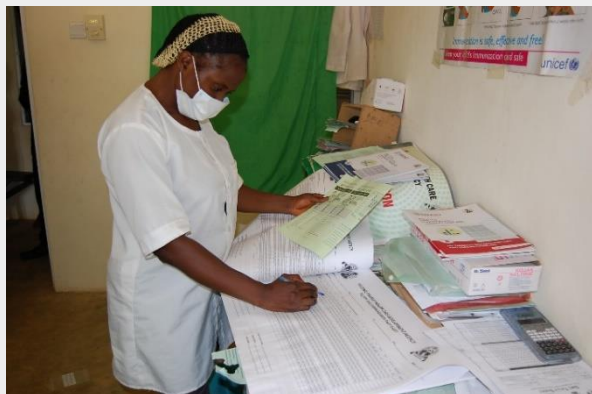
On Nutrition, they have added knowledge on Exclusive Breastfeeding, the duration for it and why it is important for every child. Complementary feeding and its examples; plumply nuts, pap, eggs, milk and the likes. Also, the right foods a pregnant woman should eat and more

On Malaria, before now they only do clinical diagnosis but now, they make good use of the RDT for diagnosis and this has helped them treat cases easily and has helped them identify complicated and less complicated Malaria cases. They learnt that IPT should be given to pregnant women as it protects them.

They also learnt, the use of the Chart Booklet as a guide and they treat based on what the booklet entails, they can easily assess & classify the sickness of a child.

They further mentioned that they use the Timer to count a child's breathing per minute and they can easily identify an abnormality in breathing. All these and more have been acquired during this training.

PHC ALJANNARE



Markus Bridget of Aljannare Health Centre spoke extensively of she wasn't using the Middle Upper Arm Circumference Tape

properly, in her words "I just place it anyhow on the arm and I didn't understand the colors but due to this training I can say I use the MUAC Tape so well and I am

excited to get the accurate results and give the right counsel".



Bala Bagudu talked about the lack of a good weighing scale at the facility and that has reduced their efficiency, he said now they use the scale properly and are happy about the intervention. He also said the Key Messaging Books are very helpful in counselling, demonstration most especially breastfeeding and supplementary feeds as there are pictures in the books that depict the rights & wrongs. On Malaria he mentioned that the weight taking helps also, and they now issue ACT alongside the right antibiotics and that not all fever should be classified as Malaria Fever but they didn't have in-depth knowledge of this before now and now with the availability of the RDT kit it is easier to detect the kind of fever.

Conclusively, the trainees demonstrated the way the use the Cold Chain, Ice Cubes, Joy style & more.

PHC BANDAM

Umar Bawa Bunza, Rabi Umar, Haruna Yakubu & Nasiru Aliyu were met as at the time of visit, they spoke highly on Child treatment



and how they have improved knowledge in those areas especially on pre-referral, respiratory issues, and the usage of the Chart Booklet.

In previous times, we refer without any form and that has made most patients not to go to the referred facilities. Thanks to the training we now refer severe cases with the referral form and even do a follow-up and we have seen changes. We administer S.P to the pregnant women and we encourage them to eat good meals for their

sound health & the baby's. we now make good use of the weighing scale, the thermometer, height ruler & the MUAC Tape, these are things we do not pay attention to before. We also study Danger signs in babies and treat them accordingly, we feel the child, assess him/her before treatment can commence.

Now, for Diarrhea we treat with Zinc & ORS, for Pneumonia we treat with Amoxicillin and ACT for Malaria. We also now know the right side to issue Antigen, the various MLs of vaccine to issue and the right time to issue.

The training has shaped us and we are esteemed to have been selected.

PHC TUNGAR MAGAJI

The following participants were present and they spoke well of how the LDHF training has improved them technically and in all aspects; Ibrahim Aliyu Koko, Ibrahim S Magaji, Shamsu Tukur & Silifatu Ibrahim.



Ibrahim said; “I must say I have improved knowledge in places I was weak such as proper counselling & addressing patients, now I counsel patients better. I can conveniently counsel a mother on how to keep clean, stay healthy and feed well. After delivery in the past we just clean them up and send them home but now we counsel them on Exclusive Breast Feeding, how to care for their babies and why they shouldn’t skip Immunization.

The others present; spoke on how they’ve gained knowledge in the use of the Child Care Card, Record card and Booklet. In the previous times, they treat based on



symptoms but now they check out for danger signs, feel the child, assess and test before treatment can be done. They also mentioned that most of their facility equipment are damaged so they do not carry out some necessities but thanks to the organizers of the training as they have new equipment and they have been taught how to properly use. While the questioning was going on a lady by name Suwaiba who was being counselled mentioned that she would not immunize her baby as her friend’s baby died as a result of wrong vaccine. She was counselled and her perception was changed.

MCH KOKO

At this facility, Sani Muhammad Koko, Aishatu Abdullahi, Aisha Sani Kele & Aishatu Umar Usman were the trainees;



Aisha Sani said “before now I

do administer low dose of Amoxicillin and Artemether but now I have learnt that the patient is to take 2 tablets and the Chart Booklet also explained in details.

They all spoke on proper use of the weighing scale, MUAC Tape, Hight Ruler and the RDT kit, they classified the young child in ranges 0-2months & 2months-5 years. They make use of the Sick Child Recording Form to enter details of the child and treat according to the Chart Booklet.

They have learnt better ways to treat children suffering from Diarrhea, Dysentery, Jaundice, Pneumonia and Malaria. They have also mastered the use of RDT for Malaria cases and how to identify Danger Signs, the main symptoms and they even track defaulters of Immunization as they now know they importance of not missing a session. They



talked about administering SP to pregnant women and exclusive breast feeding.

KOKOTOWN HEALTH POST



Before now we have been treating blindly using our experience but now, we have a guideline for treatment and refer to the

Booklet before carrying out any form of treatment. We also can easily spot danger signs such as if we see a baby convulsing, whistling and being drowsy. Now they can differentiate between difficult breathing & fast breathing and can manage these cases without hitch.



Can now develop Micro Plan for Immunization, can classify illnesses and can-do proper counselling for Caregivers on various types of Childhood Illnesses. They now give T.T to pregnant women during ANC & SP also, the Child Health should be in 3 copies, one for the Caregiver, one for the Facility & One for the Village Head. They've learnt a lot about how to identify febrile illnesses such as measles, Diarrhea as they cause High Fever.

Generally, the training has boosted their knowledge in all aspects of Child Health.

PHC K/DAMBA



Nadia Hassan, Fatima Abdullahi, Tukur Usman & Bala Muhd Jika were available for the training; they explained that they have had added knowledge on Nutrition as they can counsel mothers on Exclusive Breastfeeding and its importance, good attachment, open & close questions and healthy meals that are safe for consumption.

Secondly, they have learnt extensively on Eye & Ear treatment for children, they now give proper pre-referral treatment and refer when necessary.

They spoke on the positive impact the training has on them and more on Routine Immunization, Nutrition, Malaria, Exclusive Breastfeeding, how to use the weighing scale and measurement. They spoke highly of how they can now swap roles as a result of the training that no one is dormant in a particular job role.



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